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## FLUVIR 75 (Oseltamivir Phosphate Capsules 75mg)

## FLUVIR 45 (Oseltamivir Phosphate Capsules 45mg)

## FLUVIR 30 (Oseltamivir Phosphate Capsules 30mg)

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### NAME OF ACTIVE INGREDIENT

#### FLUVIR 75 (Oseltamivir Phosphate Capsules 75mg)

Each capsule contains 98.5mg Oseltamivir Phosphate equivalent to 75mg Oseltamivir.

#### FLUVIR 45 (Oseltamivir Phosphate Capsules 45mg)

Each capsule contains 59.1mg Oseltamivir Phosphate equivalent to 45mg Oseltamivir.

#### FLUVIR 30 (Oseltamivir Phosphate Capsules 30mg)

Each capsule contains 39.4mg Oseltamivir Phosphate equivalent to 30mg Oseltamivir.

### DOSAGE FORM

Capsules

### PRODUCT DESCRIPTIONS

#### FLUVIR 75 (Oseltamivir Phosphate Capsules 75mg):

Light yellow opaque cap / Grey opaque body size '2' hard gelatin capsules imprinted with 'H' on cap and '5' on body with blue ink, filled with white to off white granular powder.

#### FLUVIR 45 (Oseltamivir Phosphate Capsules 45mg):

Grey opaque cap / Grey opaque body size '4' hard gelatin capsules imprinted with 'H' on cap and '32' on body with blue ink, filled with white to off white granular powder.

#### FLUVIR 30 (Oseltamivir Phosphate Capsules 30mg):

Light yellow opaque cap / Light yellow opaque body size '4' hard gelatin capsules imprinted with 'H' on cap and '33' on body with blue ink, filled with white to off white granular powder.

### PHARMACODYNAMICS

Oseltamivir phosphate is a pro-drug of the active metabolite (oseltamivir carboxylate). The active metabolite is a selective inhibitor of influenza virus neuraminidase enzymes, which are glycoproteins found on the virion surface. Viral neuraminidase enzyme activity is important both for viral entry into uninfected cells and for the release of recently formed virus particles from infected cells, and for the further spread of infectious virus in the body.

Oseltamivir carboxylate inhibits influenza A and B neuraminidases *in vitro*. Oseltamivir phosphate inhibits influenza virus infection and replication *in vitro*. Oseltamivir given orally inhibits influenza A and B virus replication and pathogenicity *in vivo* in animal models of influenza infection at antiviral exposures similar to that achieved in man with 75 mg twice daily.

Antiviral activity of oseltamivir was supported for influenza A and B.

### PHARMACOKINETIC PROPERTIES

#### General Information

##### *Absorption*

Oseltamivir is readily absorbed from the gastrointestinal tract after oral administration of oseltamivir phosphate (pro-drug) and is extensively converted by predominantly hepatic esterases to the active metabolite (oseltamivir carboxylate). At least 75 % of an oral dose reaches the systemic circulation as the active metabolite. Exposure to the pro-drug is less than 5 % relative to the active metabolite. Plasma concentrations of both pro-drug and active metabolite are proportional to dose and are unaffected by co-administration with food.

##### *Distribution*

The mean volume of distribution at steady state of the oseltamivir carboxylate is approximately 23 litres in humans, a volume equivalent to extracellular body fluid. Since neuraminidase activity is extracellular, oseltamivir carboxylate distributes to all sites of influenza virus spread.

The binding of the oseltamivir carboxylate to human plasma protein is negligible (approximately 3 %).

##### *Biotransformation*

Oseltamivir is extensively converted to oseltamivir carboxylate by esterases located predominantly in the liver. Neither oseltamivir nor the active metabolite is a substrate for, or an inhibitor of, the major cytochrome P450 isoforms. No phase 2 conjugates of either compound have been identified.

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#### *Elimination*

Absorbed oseltamivir is primarily (> 90 %) eliminated by conversion to oseltamivir carboxylate. It is not further metabolised and is eliminated in the urine. Peak plasma concentrations of oseltamivir carboxylate decline with a half-life of 6 to 10 hours in most subjects. The active metabolite is eliminated entirely by renal excretion. Renal clearance (18.8 l/h) exceeds glomerular filtration rate (7.5 l/h) indicating that tubular secretion occurs in addition to glomerular filtration. Less than 20 % of an oral radiolabelled dose is eliminated in faeces.

#### Other special populations

##### *Paediatric population*

Infants less than 1 year of age: The rate of clearance of the active metabolite, corrected for body weight, decreases with ages below one year. Metabolite exposures are also more variable in the youngest infants. The available data indicates that the exposure following a 3 mg/kg dose in infants 0-12 months of age provides pro-drug and metabolite exposures anticipated to be efficacious with a safety profile comparable to that seen in older children and adults using the approved dose. The reported adverse events were consistent with the established safety profile in older children.

There are no data available for infants below 1 year of age for post exposure prevention of influenza. Prevention during an influenza epidemic in the community has not been studied in children below 12 years of age.

Post-exposure prevention of influenza in infants less than 1 year of age during a pandemic: Simulation of once daily dosing of 3mg/kg in infants < 1 year shows an exposure in the same range or higher than for once daily dosing of 75 mg in adults. Exposure does not exceed that for treatment of infants < 1 year (3 mg/kg twice daily) and is anticipated to result in a comparable safety profile. No clinical studies of prophylaxis in infants aged < 1 have been performed.

Infants and children 1 year of age or older: Younger children cleared both the pro-drug and its active metabolite faster than adults, resulting in a lower exposure for a given mg/kg dose. Doses of 2 mg/kg give oseltamivir carboxylate exposures comparable to those achieved in adults receiving a single 75 mg dose (approximately 1 mg/kg). The pharmacokinetics of oseltamivir in children and adolescents 12 years of age or older are similar to those in adults.

##### *Elderly*

Exposure to the active metabolite at steady state was 25 to 35 % higher in older people (age 65 to 78 years) compared to adults less than 65 years of age given comparable doses of oseltamivir. Half-lives observed in older people were similar to those seen in young adults. On the basis of drug exposure and tolerability, dosage adjustments are not required for older people unless there is evidence of moderate or severe renal impairment (creatinine clearance below 60 ml/min).

##### *Renal impairment*

Administration of 100 mg oseltamivir phosphate twice daily for 5 days to patients with various degrees of renal impairment showed that exposure to oseltamivir carboxylate is inversely proportional to declining renal function.

##### *Hepatic impairment*

In vitro studies have concluded that exposure to oseltamivir is not expected to be increased significantly nor is exposure to the active metabolite expected to be significantly decreased in patients with hepatic impairment.

##### *Pregnant Women*

Dose adjustments are not recommended for pregnant women in the treatment or prophylaxis of influenza.

##### *Immunocompromised Patients*

Population pharmacokinetic analyses indicate that treatment of adult and paediatric (< 18 years old) immunocompromised patients with oseltamivir results in an increased predicted exposure (from approximately 5% up to 50%) to the active metabolite when compared to non-immunocompromised patients with comparable creatinine clearance. Due to the wide safety margin of the active metabolite, no dose adjustments are required in patients due to their immunocompromised status. However, for immunocompromised patients with renal impairment, doses should be adjusted.

There was no meaningful additional benefit in exposures higher than those achieved after the administration of the standard dose.

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## INDICATION

### Treatment of influenza

Oseltamivir is indicated for the treatment of influenza in children 6 to 12 months of age during a pandemic influenza outbreak.

Oseltamivir is indicated for the treatment of influenza in adults and children  $\geq 1$  year of age.

Treatment should commence as soon as possible but no later than forty-eight hours after the onset of the initial symptoms of infection.

### Prophylaxis of influenza

Oseltamivir is indicated for post exposure prophylaxis in adults and children  $\geq 1$  year following contact with a clinically diagnosed influenza case when influenza virus is circulating in the community.

## RECOMMENDED DOSE

### **Adults, and adolescents 13 years and over**

**Treatment:** The recommended oral dose is 75 mg oseltamivir twice daily for 5 days for adolescents (13 to 17 years of age) and adults.

| Body Weight | Recommended dose for 5 days | Recommended dose for 10 days*<br>Immunocompromised Patients |
|-------------|-----------------------------|-------------------------------------------------------------|
| > 40 kg     | 75 mg twice daily           | 75 mg twice daily                                           |

\* The recommended treatment duration in immunocompromised adults and adolescents is 10 days. See *Special Populations, Immunocompromised Patients* for more information.

Treatment should be initiated as soon as possible within the first two days of onset of symptoms of influenza.

**Post-exposure prevention:** The recommended dose for prevention of influenza following close contact with an infected individual is 75 mg oseltamivir once daily for 10 days for adolescents (13 to 17 years of age) and adults.

| Body Weight | Recommended dose for 5 days | Recommended dose for 10 days<br>Immunocompromised Patients |
|-------------|-----------------------------|------------------------------------------------------------|
| > 40 kg     | 75 mg twice daily           | 75 mg twice daily                                          |

Therapy should begin as soon as possible within two days of exposure to an infected individual.

**Prevention during an influenza epidemic in the community:** The recommended dose for prevention of influenza during a community outbreak is 75 mg oseltamivir once daily for up to 6 weeks (or up to 12 weeks in immunocompromised patients, see sections Warnings and Precautions, Side Effects and Pharmacodynamics).

### **Paediatric population**

#### Children 1 to 12 years of age

**Treatment:** The following weight-adjusted dosing regimens are recommended for treatment of infants and children 1 year of age or older:

| Body Weight      | Recommended dose for 5 days | Recommended dose for 10 days*<br>Immunocompromised Patients |
|------------------|-----------------------------|-------------------------------------------------------------|
| 10 kg to 15 kg   | 30 mg twice daily           | 30 mg twice daily                                           |
| > 15 kg to 23 kg | 45 mg twice daily           | 45 mg twice daily                                           |
| > 23 kg to 40 kg | 60 mg twice daily           | 60 mg twice daily                                           |
| > 40 kg          | 75 mg twice daily           | 75 mg twice daily                                           |

\*The recommended treatment duration in immunocompromised children ( $\geq 1$  year old) is 10 days. See *Special Populations, Immunocompromised Patients* for more information.

Treatment should be initiated as soon as possible within the first two days of onset of symptoms of influenza.

**Post-exposure prevention:** The recommended post-exposure prevention dose of oseltamivir is:

| Body Weight      | Recommended dose for 10 days | Recommended dose for 10 days<br>For Immunocompromised Patients |
|------------------|------------------------------|----------------------------------------------------------------|
| 10 kg to 15 kg   | 30 mg once daily             | 30 mg once daily                                               |
| > 15 kg to 23 kg | 45 mg once daily             | 45 mg once daily                                               |
| > 23 kg to 40 kg | 60 mg once daily             | 60 mg once daily                                               |
| > 40 kg          | 75 mg once daily             | 75 mg once daily                                               |

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Prevention during an influenza epidemic in the community: Prevention during an influenza epidemic has not been studied in children below 12 years of age.

Infants 6 – 12 months of age

Treatment: The recommended treatment dose for infants 6 - 12 months of age is 3 mg/kg twice daily. This is based upon pharmacokinetic and safety data indicating that this dose in infants 6 - 12 months provides plasma concentrations of the pro-drug and active metabolite that are anticipated to be clinically efficacious with a safety profile comparable to that seen in older children and adults (see section Pharmacokinetic Properties). The following dosing regimen is recommended for treatment of infants 6 - 12 months of age:

| Body weight* | Recommended dose for 5 days | Recommended dose for 10 days**<br>Immunocompromised Patients |
|--------------|-----------------------------|--------------------------------------------------------------|
| 3 kg         | 9 mg twice daily            | 9 mg twice daily                                             |
| 4 kg         | 12 mg twice daily           | 12 mg twice daily                                            |
| 5 kg         | 15 mg twice daily           | 15 mg twice daily                                            |
| 6 kg         | 18 mg twice daily           | 18 mg twice daily                                            |
| 7 kg         | 21 mg twice daily           | 21 mg twice daily                                            |
| 8 kg         | 24 mg twice daily           | 24 mg twice daily                                            |
| 9 kg         | 27 mg twice daily           | 27 mg twice daily                                            |
| 10 kg        | 30 mg twice daily           | 30 mg twice daily                                            |

\* This table is not intended to contain all possible weights for this population. For all patients under the age of 1 year, 3 mg/kg should be used to determine dose regardless of the weight of the patient.

Treatment should be initiated as soon as possible within the first two days of onset of symptoms of influenza.

\*\* The recommended duration in immunocompromised infants (6-12 months old) is **10 days**. See *Special Populations, Immunocompromised Patients* for more information.

This dosing recommendation is not intended for premature infants, i.e., those with a post-conceptual age less than 36 weeks. Insufficient data are available for these patients, in whom different dosing may be required due to the immaturity of physiological functions.

***Special populations***

*Hepatic impairment*

No dose adjustment is required either for treatment or for prevention in patients with hepatic dysfunction. No studies have been carried out in paediatric patients with hepatic disorder.

*Renal impairment*

Treatment of influenza: Dose adjustment is recommended for adults and adolescents (13 to 17 years of age) with moderate or severe renal impairment. Recommended doses are detailed in the table below.

| Creatinine clearance          | Recommended dose for treatment             |
|-------------------------------|--------------------------------------------|
| > 60 (ml/min)                 | 75 mg twice daily                          |
| > 30 to 60 (ml/min)           | 30 mg (suspension or capsules) twice daily |
| > 10 to 30 (ml/min)           | 30 mg (suspension or capsules) once daily  |
| ≤ 10 (ml/min)                 | Not recommended (no data available)        |
| Haemodialysis patients        | 30 mg after each haemodialysis session     |
| Peritoneal dialysis patients* | 30 mg (suspension or capsules) single dose |

\*Data derived from studies in continuous ambulatory peritoneal dialysis (CAPD) patients; the clearance of oseltamivir carboxylate is expected to be higher when automated peritoneal dialysis (APD) mode is used. Treatment mode can be switched from PAD to CAPD if considered necessary by nephrologist.

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Prevention of influenza: Dose adjustment is recommended for adults and adolescents (13 to 17 years age) with moderate or severe renal impairment as detailed in the table below.

| <b>Creatinine clearance</b>   | <b>Recommended dose for treatment</b>           |
|-------------------------------|-------------------------------------------------|
| > 60 (ml/min)                 | 75 mg once daily                                |
| > 30 to 60 (ml/min)           | 30 mg (suspension or capsules) once daily       |
| > 10 to 30 (ml/min)           | 30 mg (suspension or capsules) every second day |
| ≤ 10 (ml/min)                 | Not recommended (no data available)             |
| Haemodialysis patients        | 30 mg after every second haemodialysis session  |
| Peritoneal dialysis patients* | 30 mg (suspension or capsules) once weekly      |

\*Data derived from studies in continuous ambulatory peritoneal dialysis (CAPD) patients; the clearance of oseltamivir carboxylate is expected to be higher when automated peritoneal dialysis (APD) mode is used. Treatment mode can be switched from PAD to CAPD if considered necessary by nephrologist.

There is insufficient data available in infants and children (12 years of age and younger) with renal impairment to be able to make any dosing recommendation.

#### *Elderly*

No dose adjustment is required, unless there is evidence of moderate or severe renal impairment.

#### *Immunocompromised patients*

Treatment: For treatment of influenza, the recommended duration for immunocompromised patients is 10 days. No dose adjustment is necessary. Treatment should be initiated as soon as possible within the first two days of onset of symptoms of influenza.

Seasonal prophylaxis: Longer duration of seasonal prophylaxis up to 12 weeks has been evaluated in immunocompromised patients.

### **MODE OF ADMINISTRATION**

Oral route of administration.

### **CONTRAINDICATIONS**

Oseltamivir is contraindicated in patients with known hypersensitivity to oseltamivir phosphate or to any components of the product.

### **WARNINGS AND PRECAUTIONS**

Oseltamivir is effective only against illness caused by influenza viruses. There is no evidence for efficacy of oseltamivir in any illness caused by agents other than influenza viruses.

Oseltamivir is not a substitute for influenza vaccination. Use of oseltamivir must not affect the evaluation of individuals for annual influenza vaccination. The protection against influenza lasts only as long as Oseltamivir is administered. Oseltamivir should be used for the treatment and prevention of influenza only when reliable epidemiological data indicate that influenza virus is circulating in the community.

Susceptibility of circulating influenza virus strains to oseltamivir has been shown to be highly variable. Therefore, prescribers should take into account the most recent information available on oseltamivir susceptibility patterns of the currently circulating viruses when deciding whether to use oseltamivir.

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Severe concomitant condition

No information is available regarding the safety and efficacy of oseltamivir in patients with any medical condition sufficiently severe or unstable to be considered at imminent risk of requiring hospitalisation.

Immunocompromised patients

The efficacy of oseltamivir in either treatment or prophylaxis of influenza in immunocompromised patients has not been firmly established.

Cardiac / respiratory disease

Efficacy of oseltamivir in the treatment of subjects with chronic cardiac disease and/or respiratory disease has not been established. No difference in the incidence of complications was observed between the treatment and placebo groups in this population.

Paediatric population

No data allowing a dose recommendation for premature children (< 36 weeks post-conceptual age) are currently available.

Severe renal impairment

Dose adjustment is recommended for both treatment and prevention in adolescents (13 to 17 years of age) and adults with severe renal impairment. There is insufficient clinical data available in infants and children (1 year of age or older) with renal impairment to be able to make any dosing recommendation.

Neuropsychiatric events

Neuropsychiatric events have been reported during administration of oseltamivir in patients with influenza, especially in children and adolescents. These events are also experienced by patients with influenza without oseltamivir administration. Patients should be closely monitored for behavioural changes, and the benefits and risks of continuing treatment should be carefully evaluated for each patient.

**INTERACTION WITH OTHER MEDICAMENTS**

Pharmacokinetic properties of oseltamivir, such as low protein binding and metabolism independent of the CYP450 and glucuronidase systems, suggest that clinically significant drug interactions via these mechanisms are unlikely.

Probenecid

No dose adjustment is required when co-administering with probenecid in patients with normal renal function. Co-administration of probenecid, a potent inhibitor of the anionic pathway of renal tubular secretion, results in an approximate 2-fold increase in exposure to the active metabolite of oseltamivir.

Amoxicillin

Oseltamivir has no kinetic interaction with amoxicillin, which is eliminated via the same pathway, suggesting that oseltamivir interaction with this pathway is weak.

Renal elimination

Clinically important drug interactions involving competition for renal tubular secretion are unlikely, due to the known safety margin for most of these substances, the elimination characteristics of the active metabolite (glomerular filtration and anionic tubular secretion) and the excretion capacity of these pathways. However, care should be taken when prescribing oseltamivir in subjects when taking co-excreted agents with a narrow therapeutic margin (e.g., chlorpropamide, methotrexate, phenylbutazone).

Additional information

No pharmacokinetic interactions between oseltamivir or its major metabolite have been observed when co-administering oseltamivir with paracetamol, acetylsalicylic acid, cimetidine, antacids (magnesium and aluminium hydroxides and calcium carbonates), rimantadine or warfarin (in subjects stable on warfarin and without influenza).

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## **PREGNANCY AND AND LACTATION**

### Pregnancy

Influenza is associated with adverse pregnancy and foetal outcomes, with a risk of major congenital malformations, including congenital heart defects. A large amount of data on oseltamivir exposure of pregnant women from post-marketing reports and observational studies (more than 1000 exposed outcomes during the first trimester) indicate no malformative nor fetoneonatal toxicity by oseltamivir.

The use of oseltamivir may be considered during pregnancy if necessary and after considering the available safety and benefit information and the pathogenicity of the circulating influenza virus strain.

### Breastfeeding

Limited data demonstrated that oseltamivir and the active metabolite were detected in breast milk, however the levels were low, which would result in a subtherapeutic dose to the infant. Considering this information, the pathogenicity of the circulating influenza virus strain and the underlying condition of the breastfeeding woman, administration of oseltamivir may be considered, where there are clear potential benefits to breastfeeding mothers.

## **SIDE EFFECTS**

Side effects for treatment and prevention of influenza in adults and adolescents

| <b>System Organ Class (SOC)</b>                        | <b>Adverse reactions according to frequency</b> |                                                                                                        |                                               |                                                                                                                         |
|--------------------------------------------------------|-------------------------------------------------|--------------------------------------------------------------------------------------------------------|-----------------------------------------------|-------------------------------------------------------------------------------------------------------------------------|
|                                                        | <b>Very common</b>                              | <b>Common</b>                                                                                          | <b>Uncommon</b>                               | <b>Rare</b>                                                                                                             |
| <b>Infections and infestation</b>                      |                                                 | Bronchitis,<br>Herpes simplex,<br>Nasopharyngitis,<br>Upper respiratory tract infections,<br>Sinusitis |                                               |                                                                                                                         |
| <b>Blood and lymphatic system disorders</b>            |                                                 |                                                                                                        |                                               | Thrombocytopenia                                                                                                        |
| <b>Immune system disorders</b>                         |                                                 |                                                                                                        | Hypersensitivity reaction                     | Anaphylactic reactions,<br>Anaphylactoid reactions                                                                      |
| <b>Psychiatric disorders</b>                           |                                                 |                                                                                                        |                                               | Agitation, Abnormal behaviour,<br>Anxiety, Confusion, Delusions,<br>Delirium, Hallucination,<br>Nightmares, Self-injury |
| <b>Nervous system disorders</b>                        | Headache                                        | Insomnia                                                                                               | Altered level of consciousness,<br>Convulsion |                                                                                                                         |
| <b>Eye disorders</b>                                   |                                                 |                                                                                                        |                                               | Visual disturbance                                                                                                      |
| <b>Cardiac disorders</b>                               |                                                 |                                                                                                        | Cardiac arrhythmia                            |                                                                                                                         |
| <b>Respiratory, thoracic and mediastinal disorders</b> |                                                 | Cough,<br>Sore Throat,<br>Rhinorrhea                                                                   |                                               |                                                                                                                         |
| <b>Gastrointestinal disorders</b>                      | Nausea                                          | Vomiting, Abdominal Pain (incl. upper abdominal pain),<br>Dyspepsia                                    |                                               | Gastrointestinal bleedings,<br>Haemorrhagic colitis                                                                     |
| <b>Hepatobiliary disorders</b>                         |                                                 |                                                                                                        | Elevated liver enzymes                        | Fulminant hepatitis, Hepatic failure,<br>Hepatitis                                                                      |
| <b>Skin and subcutaneous tissue disorders</b>          |                                                 |                                                                                                        | Eczema, Dermatitis,<br>Rash,<br>Urticaria     | Angioneurotic oedema, Erythema multiforme, Stevens-Johnson syndrome,<br>Toxic epidermal necrolysis                      |

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|-------------------------------------------------------------|--|-----------------------------------------------------------------------|--|--|
| <b>General disorders and administration site conditions</b> |  | Pain, Dizziness (incl. vertigo),<br>Fatigue, Pyrexia,<br>Pain in limb |  |  |
|-------------------------------------------------------------|--|-----------------------------------------------------------------------|--|--|

Side effects for treatment and prevention of influenza in children (age/weight-based dosing [30 mg to 75 mg o.d])

| System Organ Class (SOC)                               | Adverse reactions according to frequency |                                                                      |                                                       |      |
|--------------------------------------------------------|------------------------------------------|----------------------------------------------------------------------|-------------------------------------------------------|------|
|                                                        | Very common                              | Common                                                               | Uncommon                                              | Rare |
| <b>Infections and infestations</b>                     |                                          | Otitis media                                                         |                                                       |      |
| <b>Nervous system disorders</b>                        |                                          | Headache                                                             |                                                       |      |
| <b>Eye disorders</b>                                   |                                          | Conjunctivitis (including red eyes, eye discharge and eye pain)      |                                                       |      |
| <b>Ear and labyrinth disorders</b>                     |                                          | Earache                                                              | Tympanic membrane disorder                            |      |
| <b>Respiratory, thoracic and mediastinal disorders</b> | Cough,<br>Nasal congestion               | Rhinorrhoea                                                          |                                                       |      |
| <b>Gastrointestinal disorders</b>                      | Vomiting                                 | Abdominal pain (incl. upper abdominal pain),<br>Dyspepsia,<br>Nausea |                                                       |      |
| <b>Skin and subcutaneous tissue disorders</b>          |                                          |                                                                      | Dermatitis (including allergic and atopic dermatitis) |      |

### **Description of selected adverse reactions**

#### *Psychiatric disorders and nervous system disorders*

Influenza can be associated with a variety of neurologic and behavioural symptoms which can include events such as hallucinations, delirium, and abnormal behaviour, in some cases resulting in fatal outcomes. These events may occur in the setting of encephalitis or encephalopathy but can occur without obvious severe disease. In patients with influenza who were receiving Oseltamivir, there have been postmarketing reports of convulsions and delirium (including symptoms such as altered level of consciousness, confusion, abnormal behaviour, delusions, hallucinations, agitation, anxiety, nightmares), in a very few cases resulting in self-injury or fatal outcomes. These events were reported primarily among paediatric and adolescent patients and often had an abrupt onset and rapid resolution. The contribution of Oseltamivir to those events is unknown. Such neuropsychiatric events have also been reported in patients with influenza who were not taking Oseltamivir.

#### *Hepato-biliary disorders*

Hepato-biliary system disorders, including hepatitis and elevated liver enzymes in patients with influenza-like illness. These cases include fatal fulminant hepatitis/hepatic failure.

### **Other special populations**

#### *Paediatric population (infants less than one year of age)*

Pharmacokinetics, pharmacodynamics and safety profile of oseltamivir therapy in influenza infected children less than one year of age, the safety profile was similar among age cohorts with vomiting, diarrhoea and diaper rash being the most frequently reported adverse events. Insufficient data are available for infants who have a post-conceptual age of less than 36 weeks. Safety information available on oseltamivir administered for treatment of influenza in infants less than one year of age suggest that the safety profile in infants less than one year of age is similar to the established safety profile of children aged one year and older.

#### *Older people and patients with chronic cardiac and/or respiratory disease*

In general, the safety profile in the patients “at risk” was qualitatively similar to that in otherwise healthy adults/adolescents.



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*Immunocompromised patients*

The safety profile of Oseltamivir was consistent with that in Oseltamivir prophylaxis.

*Children with pre-existing bronchial asthma*

In general, the adverse reaction profile in children with pre-existing bronchial asthma was qualitatively similar to that of otherwise healthy children.

**SYMPTOMS AND TREATMENT OF OVERDOSE**

Adverse events reported following overdose were similar in nature and distribution to those observed with therapeutic doses of oseltamivir, described in section *Side Effects*.

No specific antidote is known.

Paediatric population

Overdose has been reported more frequently for children than adults and adolescents. Caution should be exercised when preparing Oseltamivir oral suspension and when describing Oseltamivir products to children.

**EFFECTS ON ABILITY TO DRIVE AND USE MACHINES**

Oseltamivir has no influence on the ability to drive and use machines.

**INSTRUCTIONS FOR USE**

Oseltamivir may be taken with or without food. However, Oseltamivir taken with food may enhance tolerability in some patients.

During situations when commercially manufactured Oseltamivir oral suspension is not readily available, adults, adolescents or children who are unable to swallow capsules may receive appropriate doses of Oseltamivir by opening capsules and pouring the contents of capsules into a suitable, small amount (1 teaspoon maximum) of sweetened food product such as regular or sugar-free chocolate syrup, honey (only for children two years or older), light brown or table sugar dissolved in water, dessert toppings, sweetened condensed milk, apple sauce or yogurt to mask the bitter taste. The mixture should be stirred and the entire contents given to the patient.

The mixture must be swallowed immediately after its preparation.

**PHARMACEUTICAL PARTICULARS**

**List of Excipients:**

FLUVIR 75 (Oseltamivir Phosphate Capsules 75mg)

Capsule content: Pregelatinized starch, Croscarmellose sodium, Povidone, Talc, Sodium Stearyl Fumarate

Capsule Cap: Iron Oxide Red, Iron Oxide Yellow, Titanium Dioxide, Gelatin (Bovine), Water

Capsule Body: Iron Oxide Black, Titanium Dioxide, Gelatin (Bovine), Water

FLUVIR 45 (Oseltamivir Phosphate Capsules 45mg)

Capsule content: Pregelatinized starch, Croscarmellose sodium, Povidone, Talc, Sodium Stearyl Fumarate

Capsule Cap: Iron Oxide Black, Titanium Dioxide, Gelatin (Bovine), Water

Capsule Body: Iron Oxide Black, Titanium Dioxide, Gelatin (Bovine), Water

FLUVIR 30 (Oseltamivir Phosphate Capsules 30mg)

Capsule content: Pregelatinized starch, Croscarmellose sodium, Povidone, Talc, Sodium Stearyl Fumarate

Capsule Cap: Iron Oxide Red, Iron Oxide Yellow, Titanium Dioxide, Gelatin (Bovine), Water

Capsule Body: Iron Oxide Red, Iron Oxide Yellow, Titanium Dioxide, Gelatin (Bovine), Water

**Storage condition**

Store below 30°C.

**Packaging available:**

FLUVIR 75 (Oseltamivir Phosphate Capsules 75mg): 3 x 10s

FLUVIR 45 (Oseltamivir Phosphate Capsules 45mg): 3 x 10s

FLUVIR 30 (Oseltamivir Phosphate Capsules 30mg): 3 x 10s

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**FLUVIR 75 (Oseltamivir Phosphate Capsules 75mg)**  
**FLUVIR 45 (Oseltamivir Phosphate Capsules 45mg)**  
**FLUVIR 30 (Oseltamivir Phosphate Capsules 30mg)**

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**Manufactured by:**

Hetero Labs Limited,  
Unit-III, #22 – 110, I.D.A.,  
Jeedimetla, Hyderabad - 500 055, Telangana, India

**Product Registration Holder:**

Camber Laboratories Sdn. Bhd.,  
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